Hospital Discharge Process Improvement

Business Requirements Document

Prepared By: Samantha Bradley-Holt

Role: Business Analyst

Date: March 2026

Version 1.0

Table of Contents

1. Executive Summary 2
2. Business Case 3
2.1 Project Objectives 3
3. Project Scope 3
3.1 In Scope 3
3.2 Out of Scope 4
4. Stakeholder Analysis 4
4.1 Stakeholder Identification 4
4.2 Stakeholder Roles and Responsibilities 4
4.3 Stakeholder Influence Matrix 5
4.4 Stakeholder Communication Plan 5
5. Current State (AS-IS Process) 6
5.1 Description 7
5.2 AS-IS Flowchart 7
6. Pain Points and Gap Analysis 7
6.1 Pain Points 8
6.2 Gaps 8
7. Future State (TO-BE Process) 8
7.1 Description 9
7.2 TO-BE Flowchart 9
7.3 Addressed Pain Points 9
7.4 Key Benefits Alignment 10
8. Expected Benefits 10
8.1 Operational and Patient Care Benefits 11
8.2 Additional Strategic Benefits 11
9. Functional Requirements 11

10. Use Cases 12
10.1 UC-01 Identify High-Risk Patients 13
10.2 UC-02 Social Worker Intervention 13
10.3 UC-03 Schedule Follow-Up Appointment 13
10.4 UC-04 Provide Discharge Education 14
10.5 UC-05 Deliver Standardized Discharge Education 14
10.6 UC-06 Track Post-Discharge Outcomes 15
11. Non-Functional Requirements 16
12. Success Metrics (KPI's) 17
13. Risks and Assumptions 17
13.1 Risks 18
13.2 Assumptions 18
14. Implementation Considerations 18
15. Requirements Traceability Matrix 19

1. Executive Summary

Hospitals are experiencing elevated 30-day readmission rates, exceeding national benchmarks. This results in financial penalties, operational inefficiencies, and reduced patient satisfaction. The Hospital Readmission Reduction Initiative aims to identify the key drivers of readmissions, improve discharge and follow-up processes, and provide actionable insights through a reporting dashboard. These insights will enable clinical staff to identify characteristics of demographics that are potentially at risk of being readmitted.

2. Business Case

The hospital currently has a 30-day readmission rate of 15%, which is above the national benchmark of 14%. Contributing factors include inconsistent follow-up appointment scheduling, limited visibility into high-risk patients, and a lack of standardized discharge procedures. These issues increase operational costs and negatively affect patient outcomes.

2.1 Project Objectives

Objective ID	Objective
OBJ-1	Reduce 30-day readmission rate from 18% to 12% within 12 months
OBJ-2	Ensure 90% of discharged patients have follow-up appointments scheduled
OBJ-3	Provide leadership with real-time dashboards to monitor readmission trends

3. Project Scope

3.1 In Scope

- Analysis of the current hospital discharge process
- Identification of patient readmission risk factors
- Implementation of a standardized discharge risk assessment
- Coordination of follow-up appointments prior to discharge
- Identification of barriers to medication access (transportation, delivery options)

3.2 Out of Scope

- Changes to physician treatment decisions
- Insurance policy or coverage modifications
- Hospital billing or financial operations
- Development of new clinical treatment protocols
- Hospital-wide IT infrastructure upgrades unrelated to readmission analysis

4. Stakeholder Analysis

4.1 Stakeholder Identification

Stakeholder ID	Stakeholder	Department
STK-01	Hospital Administration	Executive
STK-02	Physicians	Clinical
STK-03	Medical Social Worker	Care Coordination
STK-04	Nursing Staff	Patient Care
STK-05	IT Department	Technology
STK-06	Quality Improvement Team	Compliance
STK-07	Patients	Healthcare Recipients

4.2 Stakeholder Roles and Responsibilities

Stakeholder	Role in Project
Hospital Administration	Project sponsors and decision-makers
Physicians	Provide clinical insight into discharge processes
Medical Social Worker	Identify patient support needs
Nursing Staff	Manage patient discharge and follow-up instructions
IT Department	Provide data access and support dashboard implementation
Quality Improvement Team	Monitor readmission metrics and regulatory compliance
Patients	Represent the end users of the discharge process; provide perspective on patient barriers, medication access, and post-discharge support.

4.3 Stakeholder Influence Matrix

Stakeholder	Influence	Interest	Engagement Strategy
Hospital Administration	High	High	Regular status updates and dashboard reports
Physicians	Medium	High	Consult during workflow improvement discussions
Medical Social Worker	Medium	High	Manage social risk factors
Nursing Staff	Medium	High	Include in process design workshops
IT Department	High	Medium	Coordinate technical requirements and reporting tools
Quality Improvement Team	Medium	High	Collaborate on readmission metrics and compliance tracking
Patient	Low	High	Ensure patients understand post-charge instructions Collect feedback on barriers such as transportation, medication access, support at home Provide support options where needed (pharmacy delivery, home health services, caregiver coordination)

4.4 Stakeholder Communication Plan

Stakeholder	Communication Method	Frequency
Hospital Administration	Executive reports / dashboard updates	Monthly
Physicians	Process improvement meetings	As needed
Medical Social Worker	Process improvement meetings	As needed
Nursing Staff	Workflow workshops and training	During implementation
IT Department	Technical planning meetings	Weekly during development
Quality Improvement Team	Metrics review sessions	Monthly
Patients	Direct education; discharge instructions; and nurse guidance	Each discharge

5. Current State (AS-IS Process)

5.1 Description

The current discharge process begins when a physician determines that a patient is medically stable and ready to leave the hospital. Nursing staff prepare discharge instructions, provide medication guidance, and release the patient from the hospital.

Follow-up care is sometimes scheduled prior to discharge, but this process is not standardized. As a result, some patients leave without a scheduled follow-up appointment or without clear post-discharge care coordination. This lack of consistency can contribute to avoidable hospital readmissions within 30 days.

5.2 AS-IS Flowchart

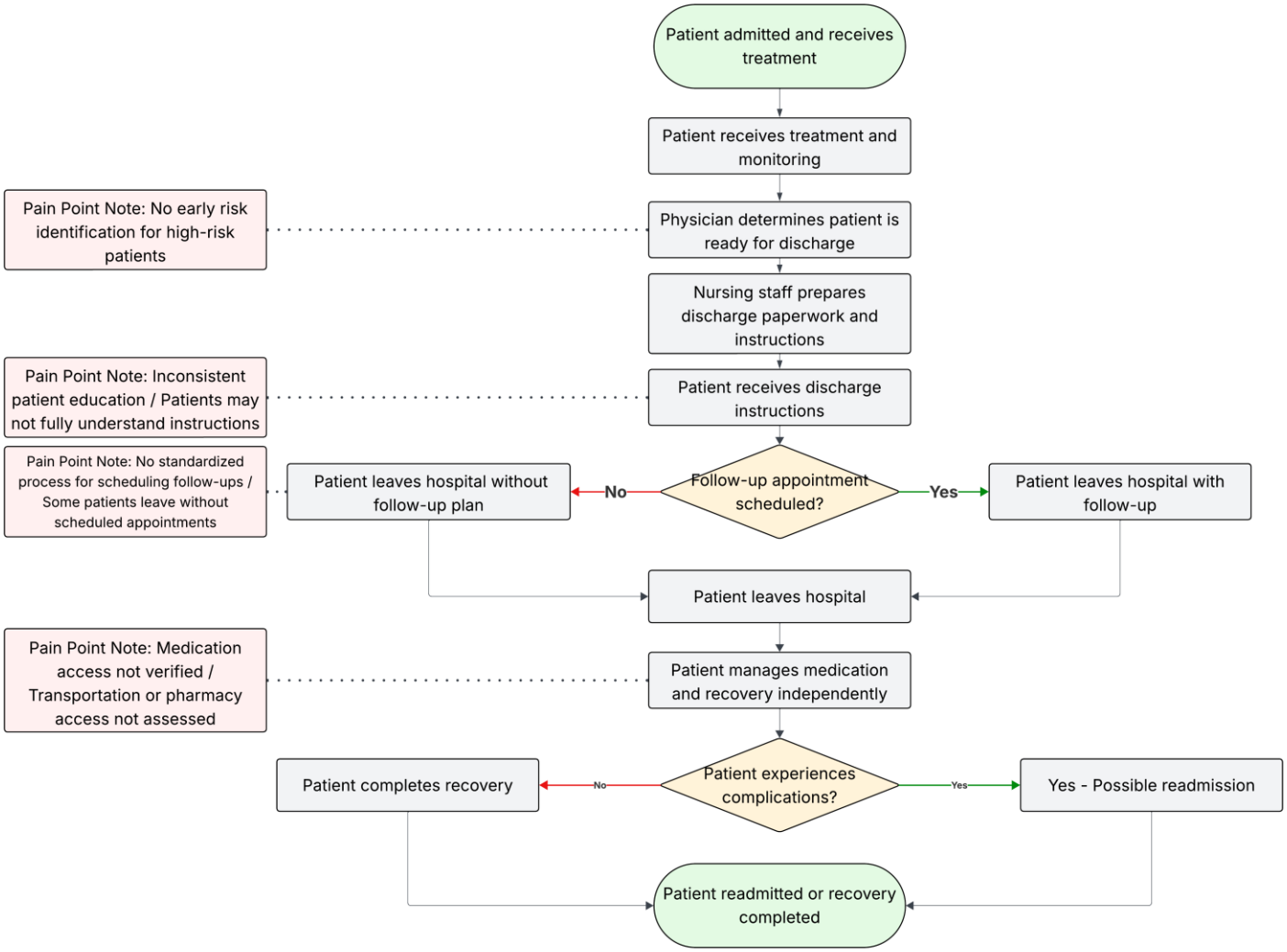

Figure 1. Current Hospital Discharge Process (AS-IS)

6. Pain Points and Gap Analysis

6.1 Pain Points

Pain Point Identified	AS-IS Process
Inconsistent patient education	Discharge instructions vary between nursing staff, leaving patients unclear on recovery steps.
Medication access barriers	Patients may lack transportation or technical capability to obtain prescribed medications.
Follow-up coordination gaps	Scheduling follow-up appointments is often left to patients, leading to missed visits.
Delayed risk identification	High-risk patients are sometimes only flagged at discharge, limiting intervention opportunities.

6.2 Gaps

- No standardized risk assessment during treatment or prior to discharge.
- Limited social worker involvement for high-risk patients.
- Inconsistent discharge communication across staff and departments.
- Insufficient use of technology for follow-up scheduling and medication delivery options.

These pain points and process gaps contribute to avoidable 30-day readmissions, inefficient resource use, and inconsistent patient experiences. Addressing them is critical to improving patient outcomes and streamlining hospital operations.

7. Future State (TO-BE Process)

7.1 Description

The improved hospital discharge process addresses the current gaps by standardizing risk assessments, involving social worker consultation for high-risk patients, and ensuring consistent patient education. Follow-up appointments are scheduled proactively before discharge, and medication access barriers are addressed through pharmacy delivery or technology-assisted ordering. These enhancements aim to reduce readmissions, improve patient outcomes, and streamline care coordination.

7.2 TO-BE Flowchart

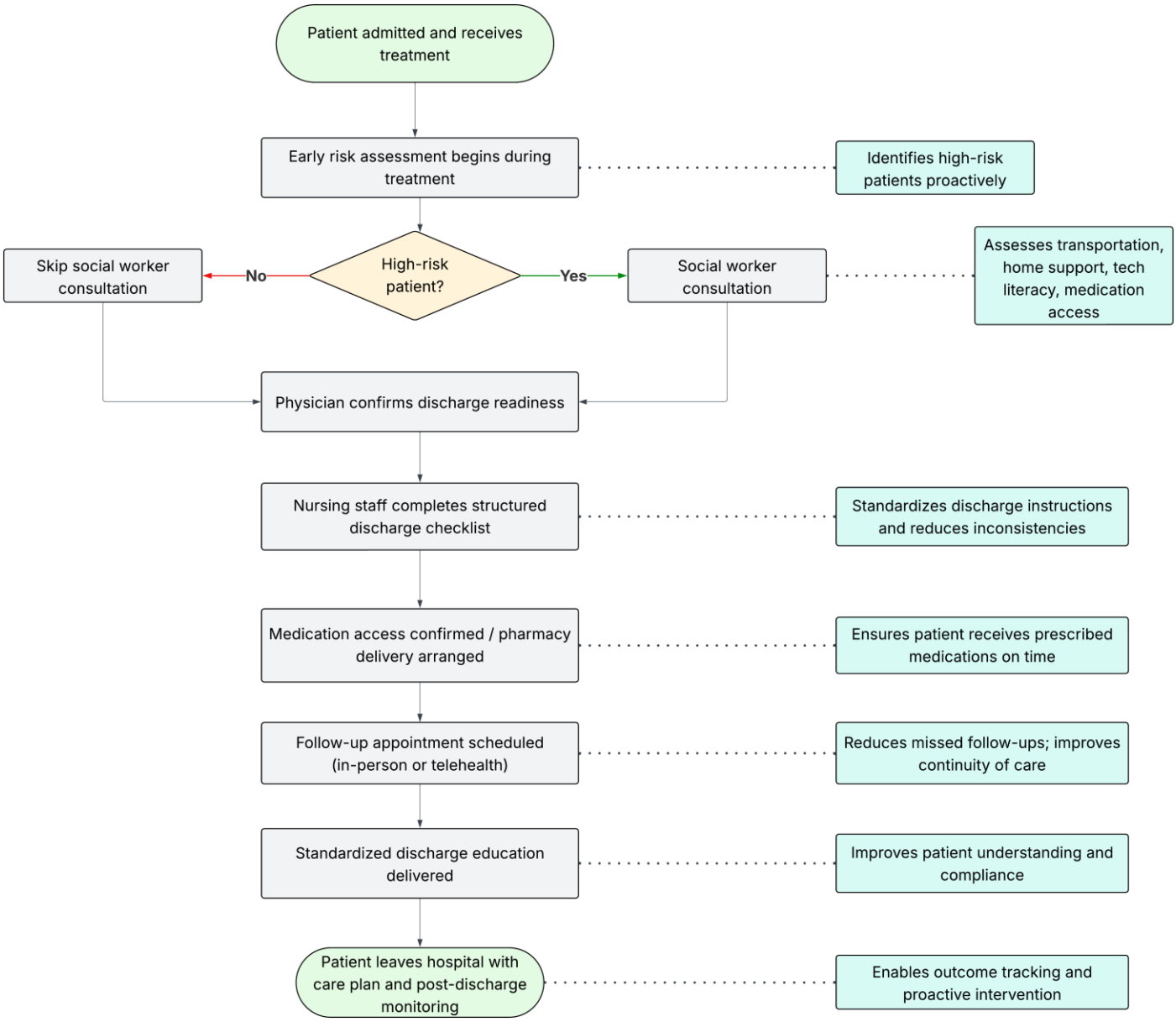

Figure 2 – Improved Hospital Discharge Process (TO-BE)

7.3 Addressed Pain Points

Pain Point	TO-BE Solution
Inconsistent patient education	Standardized discharge instructions for all patients, ensuring clarity and consistency.
Medication access barriers	Pharmacy delivery options and technology-assisted medication ordering.
Follow-up coordination gaps	Follow-up appointments are scheduled prior to discharge with clear reminders.
Delayed risk identification	Risk assessment conducted during treatment, allowing proactive intervention for high-risk patients.
Limited social worker involvement	Automatic referral to social worker consultation for high-risk patients.

7.4 Key Benefits Alignment

TO-BE Improvement	Related Expected Benefit
Standardized patient education	Improved patient understanding of care instructions
Pharmacy delivery & technology-assisted medication ordering	Better access to medications
Pre-scheduled follow-up appointments with reminders	Improved follow-up compliance
Early risk assessment during treatment	Reduced hospital readmissions
Automatic referral to social worker for high-risk patients	Enhanced care coordination

8. Expected Benefits

The improved process promotes efficiency and ensures the patient is being discharged home with clear instructions that they understand and have a plan for continued recovery at home.

8.1 Operational and Patient Care Benefits

Benefit Category	Expected Benefit	Measurement / KPI
Patient Outcomes	Reduction in 30-day hospital readmissions through early risk identification and targeted interventions	30-day readmission rate
Operational Efficiency	Standardized discharge workflow reduces inconsistencies across departments	Average discharge processing time
Care Coordination	Improved collaboration between physicians, nursing staff, social workers, and pharmacy	Number of coordinated discharge interventions
Medication Adherence	Medication delivery coordination reduces missed or delayed prescriptions	Prescription pickup or delivery completion rate
Access to Follow-Up Care	Telehealth follow-up options improve appointment attendance for patients with transportation or mobility barriers	Follow-up appointment attendance rate
Patient Experience	Clear discharge instructions and education increase patient confidence in post-discharge care	Patient satisfaction survey results
Data Visibility	Dashboard reporting enables leadership to monitor readmission trends and evaluate interventions	Readmission analytics dashboard usage

8.2 Additional Strategic Benefits

- Improved patient recovery outcomes
- Reduced avoidable hospital costs associated with readmissions
- Better compliance with quality care standards
- Increased hospital reputation and patient trust
- Improved data-driven decision making for leadership

9. Functional Requirements

ID	Requirement	Description	Associated TO-BE Step	Priority	Responsible Stakeholder
FR-01	Early Risk Assessment	System flags high-risk patients during treatment and alerts care team based on clinical and social factors.	Early risk assessment begins during treatment	High	Physician, Nurse
FR-02	Social Worker Access	Social worker can view patient risk info and document interventions	Social worker consultation	High	Social Worker
FR-03	Standardized Discharge Checklist	Nurses follow structured checklist for discharge	Nursing staff completes checklist	Medium	Nurse
FR-04	Medication Coordination	System verifies patient can access medications and coordinates pharmacy delivery if needed	Medication access confirmed / pharmacy delivery arranged	High	Nurse, Pharmacy
FR-05	Follow-Up Scheduling	Schedule follow-up appointments (in-person or telehealth) for all patients	Follow-up appointment scheduled	High	Nurse, Scheduling Staff
FR-06	Discharge Education	Provide standardized discharge instructions, including medication and care plan	Standardized discharge education delivered	Medium	Nurse
FR-07	Patient Communication	Notify patients of follow-up appointments and medication delivery	Follow-up appointment scheduled / medication access confirmed	Medium	Nurse, Social Worker
FR-08	Outcome Tracking	Track post-discharge outcomes and readmission risk	End – patient leaves hospital with care plan & monitoring	Medium	Physician, Quality Team
FR-09	Decision Support Alerts	Notify care team of any patients missing critical discharge steps	Nursing staff completes checklist / medication coordination	High	Nurse, Physician
FR-10	Documentation Logging	Record all interventions, risk assessments, and follow-up scheduling in system	Record all interventions, risk assessments, and follow-up scheduling in system All TO-BE steps	High	Nurse, Social Worker, Physician

10. Use Cases

The following use cases describe how stakeholders interact with the improved discharge process to support patient care coordination and reduce readmission risk.

10.1 UC-01 Identify High-Risk Patients

Field	Description
Use Case ID	UC-01
Use Case Name	Identify High Risk Patients
Primary Actor	Physician / Nurse
Secondary Actors	Hospital system
Description	Patient risk factors are evaluated during treatment to identify potential readmission risk
Preconditions	Patient admitted and receiving treatment
Postconditions	Patient risk status recorded in system
Main Flow	1. Provider reviews patient health indicators 2. Provider enters patient data into system 3. System evaluates risk criteria 4. System flags patient as high or low risk
Alternate Flow	Risk criteria unavailable → manual review performed

10.2 UC-02 Social Worker Intervention

Field	Description
Use Case ID	UC-02
Use Case Name	Social Worker Risk Assessment
Primary Actor	Social Worker
Secondary Actors	Nurse, Patient
Description	Social worker evaluates barriers that may impact recovery after discharge
Preconditions	Patient identified as high risk
Postconditions	Support interventions documented
Main Flow	1. Social worker reviews patient risk information 2. Social worker meets patient 3. Transportation, home support, and medication access assessed 4. Intervention plan documented
Alternate Flow	Patient unavailable – follow-up scheduled

10.3 UC-03 Schedule Follow-Up Appointment

Field	Description
Use Case ID	UC-03
Use Case Name	Schedule Follow-Up Care
Primary Actor	Nurse / Scheduling Staff
Secondary Actors	Patient
Description	Follow-up appointment arranged before patient discharge
Preconditions	Physician approves discharge
Postconditions	Appointment scheduled and patient notified
Main Flow	1. Nurse accesses scheduling system 2. Available appointments reviewed 3. Patient chooses in-person or telehealth option 4. Appointment scheduled and confirmation provided
Alternate Flow	Patient unavailable for scheduled time → alternate time offered

10.4 UC-04 Provide Discharge Education

Field	Description
Use Case ID	UC-04
Use Case Name	Deliver Discharge Instructions
Primary Actor	Nurse
Secondary Actors	Patient
Description	Patient receives standardized instructions for post-discharge care
Preconditions	Discharge approved and checklist completed
Postconditions	Patient understands care plan
Main Flow	1. Nurse reviews discharge checklist 2. Medication instructions explained 3. Follow-up appointment reviewed 4. Patient confirms understanding
Alternate Flow	Patient requests clarification → additional education provided

10.5 UC-05 Deliver Standardized Discharge Education

Field	Description
Use Case ID	UC-05
Use Case Name	Deliver Standardized Discharge Education
Primary Actor	Nurse
Secondary Actors	Patient
Description	The nurse provides standardized discharge instructions to ensure the patient understands medication use, follow-up care, and recovery requirements before leaving the hospital.
Preconditions	Physician has approved patient discharge, and the discharge checklist is initiated.
Postconditions	Patient receives discharge instructions and confirms understanding of the care plan.
Main Flow	1. Nurse reviews the patient discharge checklist. 2. Nurse retrieves standardized discharge education materials. 3. Nurse explains medication instructions, including dosage and timing. 4. Nurse reviews follow-up appointment details with the patient. 5. Nurse explains warning signs or symptoms requiring medical attention. 6. Nurse asks the patient to confirm understanding of the care plan. 7. Nurse documents completion of discharge education in the system.

Alternate Flow	• Patient does not understand instructions
	  ○ Nurse provides additional explanation or educational materials.
	• Nurse confirms understanding before discharge.
	  ○ Language barrier present
	  ○ Interpreter services or translated materials are used.

10.6 UC-06 Track Post-Discharge Outcomes

Field	Description
Use Case ID	UC-06
Use Case Name	Track Post-Discharge Outcomes
Primary Actor	Quality Improvement Team
Secondary Actors	Physician, Nurse, Hospital System
Description	The healthcare organization monitors patient outcomes after discharge to identify complications or potential readmission risks.
Preconditions	Patient has been discharged and follow-up care plan is documented.
Postconditions	Patient outcomes are tracked and any risk indicators are flagged for intervention.
Main Flow	1. System records discharge information and follow-up appointment details. 2. Patient attends scheduled follow-up visit or telehealth appointment. 3. Provider reviews patient recovery status. 4. System logs patient outcomes and updates monitoring records. 5. Quality team reviews outcome data and identifies potential readmission risks.
Alternate Flow	1. Missed follow-up appointment 2. System flags missed appointments. 3. Care team contacts patients to reschedule. 4. Complication detected during follow-up 5. Physician initiates additional treatment or monitoring.

11. Non-Functional Requirements

ID	Requirement Type	Requirement Description	Priority
NFR-01	Security	Patient health information must comply with HIPAA regulations and be protected from unauthorized access	High
NFR-02	Access Control	Only authorized hospital staff (physicians, nurses, social workers) may access patient discharge data	High
NFR-03	System Availability	The system must be available to hospital staff during operational hours with minimal downtime	High
NFR-04	Performance	Patient risk assessments and alerts must be generated within 5 seconds of data entry	Medium
NFR-05	Usability	Discharge checklists and patient information screens must be simple and easy for clinical staff to complete quickly	Medium
NFR-06	Data Accuracy	The system must ensure accurate recording of patient discharge information and interventions	High
NFR-07	Audit Logging	All patient interactions and system updates must be logged for compliance and auditing purposes	Medium
NFR-08	Reliability	The system must maintain data integrity and prevent loss of patient information	High
NFR-09	Scalability	The system should support increasing numbers of patient records without performance degradation	Low
NFR-10	Interoperability	The system should integrate with hospital electronic health record systems and scheduling platforms	Medium

12. Success Metrics (KPI's)

KPI	Description	Target
Readmission Rate	% of patients readmitted within 30 days Reduce by 10–20%	Reduce by 10–20%
Follow-Up Compliance	% of patients attending scheduled follow-up visits	90%
Medication Access Confirmation	% of patients with verified medication access before discharge	95–100%
High-Risk Patient Intervention	% of high-risk patients receiving social worker intervention	100%
Discharge Checklist Completion	% of discharges with completed checklist	95%

13. Risks and Assumptions

This section identifies potential risks that may impact the successful implementation of the proposed discharge process improvements, as well as key assumptions made during the analysis.

13.1 Risks

Risk ID	Risk Description	Impact	Mitigation Strategy
R-01	Patients may not attend scheduled follow-up appointments	Higher risk of readmission	Implement appointment reminders and telehealth options
R-02	Staff may resist adopting new discharge processes	Reduced compliance with new workflow	Provide training and clear discharge checklists
R-03	Patients may lack transportation or support at home	Delays in medication pickup or follow-up care	Social worker assessment to identify and address barriers
R-04	Technology integration issues with existing hospital systems	Delays in implementation	Conduct system compatibility testing before rollout
R-05	Incomplete documentation by staff	Gaps in patient discharge information	Use mandatory checklist completion before discharge

13.2 Assumptions

Assumption ID	Assumption Description
A-01	Hospital staff will receive training on the new discharge workflow
A-02	The hospital has an electronic health record system capable of supporting discharge documentation
A-03	Social workers are available to assess high-risk patients
A-04	Patients have access to either in-person or telehealth follow-up appointments
A-05	Pharmacy services can support medication delivery when needed

14. Implementation Considerations

- Workflow training for nurses
- EHR integration
- Social Worker availability
- Dashboard deployment

15. Requirements Traceability Matrix

Business Objective	Functional Requirement	Use Case	TO-BE Process Step
Reduce hospital readmissions	FR-01 Early Risk Assessment	UC-01 Identify High-Risk Patient	Early risk assessment during treatment
Reduce hospital readmissions	FR-02 Social Worker Access	UC-02 Social Worker Risk Assessment	Social worker consultation
Ensure patients receive medications	FR-04 Medication Coordination	UC-03 Confirm Medication Access	Medication access confirmed / pharmacy delivery arranged
Improve continuity of care	FR-05 Follow-Up Scheduling	UC-04 Schedule Follow-Up Appointment	Follow-up appointment scheduled
Improve patient understanding of care plan	FR-06 Discharge Education	UC-05 Deliver Discharge Education	Standardized discharge education delivered
Monitor patient outcomes	FR-08 Outcome Tracking	UC-06 Track Post-Discharge Outcomes	Post-discharge monitoring